The Hillingdon Hospitals
NHS Foundation Trust

Maternity and Neonatal Debrief Pathway

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Version	Author	Summary of changes

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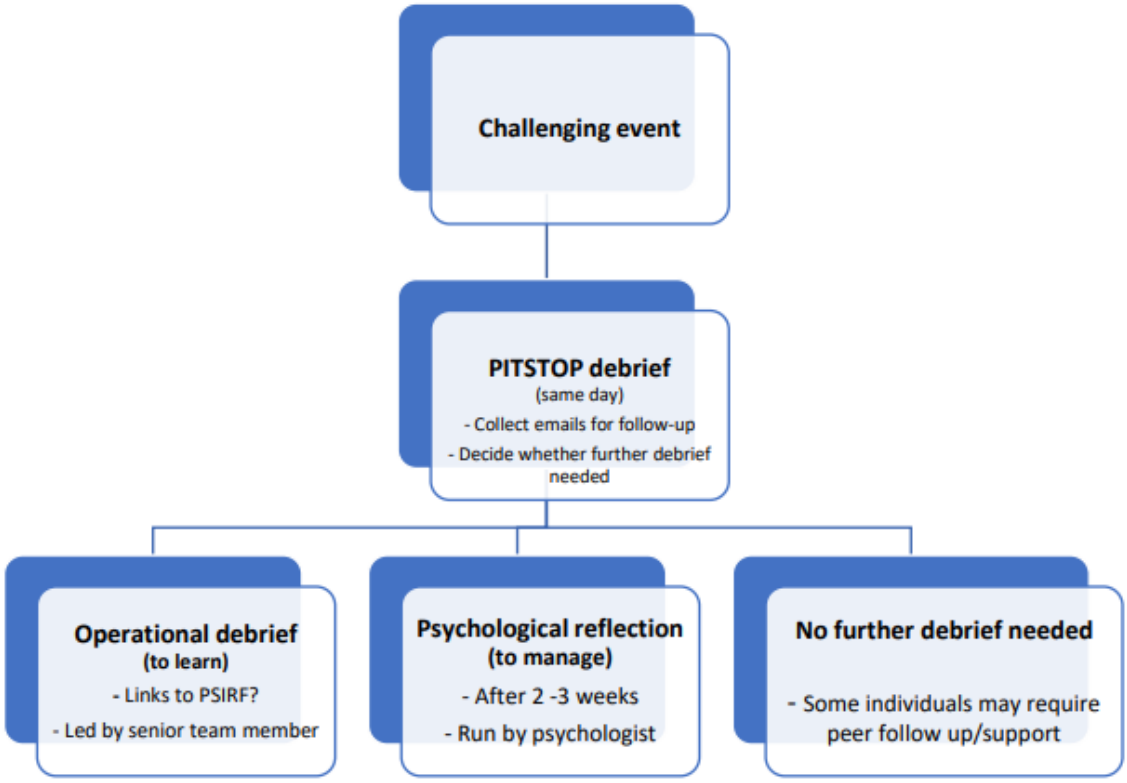
Aim/Purpose

This guideline provides a suggested pathway to facilitate **cold debriefs**. It is not intended to replace any pre-existing support and is simply to be considered as an addition, and not a substitute. Whilst not the focus of this guideline: staff psychological wellbeing should be paramount and therefore it is ideal that hot debriefs are also utilised as soon as possible following the event. The PITSTOP structure is a useful format by which to conduct this (See Appendix 4)

Introduction

Debriefing is the process of communication that takes place between a team, or multiple teams, following a clinical case. There are 2 main types of debriefs: 'hot' debriefs i.e. there and then, as the people involved are likely to still be on shift, and 'cold' debriefs which can happen any time after the day following the event. Cold debriefs can be further broken down into 'psychological debriefs' i.e. a post event team reflection, or 'operational debriefs' with an aim to learn and enact subsequent change.

Figure 1



The Guidance

What

A debrief is a structured way of enabling teams to process an experience and as such, a suggested template has been attached to this guideline (please see Appendix 1).

The general principles are:

- **Confidentiality:** nothing from the debrief is documented in medical notes. Whilst this is not a governance meeting, any events that come up that need any actions, will be communicated with the clinical leads involved in the case.
- **Support:** as well as the original event itself, a discussion regarding a traumatic event can bring up complex feelings, and so the debrief facilitator may contact a member of the team afterwards to ensure they are being supported and help signpost them to resources (please see Appendix 2).
- **No judgement:** hierarchy and 'ranks' are left at the door with all voices valued, and everyone should be able to interact in any way they feel comfortable e.g. sharing if they wish to do so or listening to others speak.

The purpose of an operational debrief to a psychological debrief differs, with the former having a focus on shared learning and action planning and the latter to promote psychological wellbeing and foster team belonging. As such there must be careful consideration given the aims and purpose of the debrief prior to carrying it out by the facilitator. All team members involved should be made aware of this at the beginning of the session to make clear staff expectations.

The table below summarises the main differences between these types of cold debrief.

Figure 2

Operational Clinical Debrief (to learn)	Psychological Debrief (to manage)
• Create safe learning context • Share initial reactions • Clarify facts • Explore performance domains • Identify take-aways	• Do not explore experiences in details • Provide space • Acknowledge various reactions • Signpost to support • Identify positives

Adapted from Kolbe et al. (2021)

Why

Since its introduction in the aviation industry, debriefing is now thought to be an essential practice within healthcare, with a wide range of benefits listed below.

- Improves patient safety, clinical practice and teamwork and communication within the MDT.
- Facilitates identification and thereby rectification of issues e.g. staffing shortages, system performance, equipment failure, and can help address training needs and gaps in knowledge, alongside guiding new quality improvement projects.
- Aims to safeguard staff wellbeing and helps reduce the impact of stress, and can help identify adverse effects, such as rumination and feelings of guilt, and appropriately signpost to further support.
- A study on hot debriefs showed that 100% felt the process helped with their clinical practice, and 90% felt they benefited psychologically from the process.
- Significant events whilst rare, can have long lasting impacts on staff. As an example, studies have shown that paediatric trainees are at a high risk of developing acute stress reactions (ASR) and PTSD following child death.
- In one study from 2018, 203/251 (81%) of Paediatric trainees reported one or more symptoms or behaviours that could contribute to a diagnosis of ASR/PTSD.

When

A debrief can take place after **any event** that staff would like to discuss further. Examples include but are not exhaustive:

- Cardiorespiratory arrest or prolonged resuscitation
- After severe obstetric emergencies regardless of outcome
- Maternal ITU admission or severe deterioration
- Maternal or neonatal death
- Hypoxic ischaemic encephalopathy or unwell neonate requiring transfer out
- NAI or safeguarding concerns

Who

The cold debrief can include **anyone involved** in the case, as all may benefit from talking about an incident. It is an opportunity for individuals to ask questions about what happened and discuss their feelings, and different teams may have different insights based on experience from their respective organisations. While everyone should be invited, attendance is not mandatory and is an optional practice.

Examples of teams to invite to the debrief may include (but are not limited to):

- Pre-hospital teams e.g. London Ambulance Service or homebirth team
- All clinical staff across various MDT services, as well as any respective students present.
- Specialist teams e.g. surgeons, anaesthetists, intensivists, radiology, ultrasound

- Support and clerical teams e.g. porters, admin staff, lab technicians
- Transport teams e.g. Neonatal Transport Service or Children's Acute Transport Service.
- External teams in receiving unit e.g. tertiary NICU or ICU staff.

Remember this is not an exhaustive list and there may be other staff groups to include in the debrief.

How

A debrief may be led by anyone, however it can be useful to have someone who wasn't present during the event and depending on the type of debrief someone who has received relevant training in facilitation of debriefs. For an operational debrief this may include someone from the governance team or senior leadership. For a psychological debrief this may include a clinical psychologist.

To help capture staff present, staff names and emails should ideally be **taken at the hot debrief**, however if this is not possible, please raise this with Designated Debrief Leads (DDLs*) via the below form who can then organise this

*DDLs are permanent members of staff who have nominated themselves to help facilitate this pathway

To contact a DDL and request a debrief, please scan the QR code or visit https://forms.office.com/e/VY0GRDTSQC

What next

Once a request for a debrief has been received, the DDLs will coordinate, liaise with the psychology team if required and aim to collate names within the next week, and then arrange a cold debrief the week after that if possible. The aim is for all debriefs to take place within 3 weeks from the event.

The DDLs will then send out an MS teams invite such that the debrief can be a hybrid meeting, so that staff may attend either in-person or online. It may be necessary to arrange more than one debrief session in order to allow as many staff who wish to, to attend. Please see Appendix 1 and Appendix 4 for examples of frameworks which can be used to facilitate both hot and cold debriefs.

If it is felt that alternative support should be offered in lieu of a debrief, the DDLs will get back in contact with the people involved in the incident.

Associated Guidelines, Polices and Documents

None.

Training Need Identified (if yes, details)

None.

Implementation and Monitoring

Clinical guidelines are available to staff via the Trust's Document Information Management System (DiMS). All staff have a responsibility to ensure that they are referring to the most recent version of the document. A link to any new/updated clinical guidelines which have been published within the last 3 months is included in the Trust-e-News publication on a weekly basis.

There will be a link to keep an anonymous record of the fact that a debrief has taken place, an anonymous brief summary, and any learning for the process of debrief.

References

- Hollingsworth CE, Wesley C, Huckridge J, et al. Arch Dis Child 2018;103:14–18
- Gillen, J., Koncicki, M.L., Hough, R.F. et al. The impact of a fellow-driven debriefing program after pediatric cardiac arrests. BMC Med Educ 19, 272 (2019)
- Gilmartin S, Martin L, Kenny S, et al. Promoting hot debriefing in an emergency department. BMJ Open Quality 2020;9:e000913.
- Coggins, A., Santos, A.D.L., Zaklama, R. et al. Interdisciplinary clinical debriefing in the emergency department: an observational study of learning topics and outcomes. BMC Emerg Med 20, 79 (2020).

Appendices

Appendix 1 – an example of debriefing tool to help guide cold debrief facilitators

Appendix 2: Local resources

Appendix 3 – Links to additional resources for staff facilitating debriefs

Appendix 4 – PITSTOP debriefing tool for Hot Debriefs

Appendix 1 – an example of debriefing tool to help guide cold debrief facilitators

1) What happened?
- Would the group like to start off by discussing who arrived first?
- Offer a chance to speak about what happened to establish a timeline but remind them there is no pressure to share

2) How did it make you feel?
- What were you thinking at the time? How did you feel immediately afterwards/now?
- Is there anything you want to explore e.g. a comment, a mental image, an emotion
- Remember that sharing experiences should be voluntary, and it is advised not to request detailed sensory descriptions of traumatic events

3) What helped or hindered the situation?
- Is there anything positive that a team or an individual can take away?
- Is there anything you wish had happened and what might have stopped this from happening?

4) What next?
- Are there any clinical learning points and is there an **action plan?** (note a summary and include who is taking responsibility for this)
- Moving forwards, how might this affect you? Do you want to meet again?
- Signpost resources for help – e.g. educational supervisor, staff wellbeing page on intranet, trust psychologist, NWL Keeping Well, NHS Practitioner Health, Professional Support Unit

Appendix 2: Local resources

We would advise contacting your supervisor and/or line manager in the first instance, as they can help support you and consider modifying your work pattern if needed, as well as supporting with writing an account of events if requested e.g. as part of an MNSI investigation.

You can also access occupational health and a trust psychologist for individual support. Details can be found on the hospital intranet which has a page dedicated to staff wellbeing: http://intranetthh.hilldomain.thh.nhs.uk/index.php/wellbeing-hub

There is also the North West London Keeping Well staff support hub to which you can self refer https://www.keepingwellnwl.nhs.uk/

The Professional Midwifery Advocate (PMA) Team are a resource available to staff for psychological support following adverse events, be that personal or professional in nature. They support midwives and wider maternity staff to deliver safe high-quality care, drive quality improvement and focusing on staff wellbeing, reducing stress & burnout. Staff can access a PMA on a 1:1 or group appointment-only basis for supportive debrief, personal action planning, open & honest feedback and reflection guidance. PMA's are sessional and are based within the clinical areas; the team can be accessed personally by drop in or on appointment basis by emailing: thh.hillingdonlisteningservice@nhs.net

External resources

If you would prefer to seek additional support external to your place of work, there are a large number of resources available. Please see below some links to external organisations in addition to the local hospital support.

- NHS Practitioner Health: visit www.practionerhealth.nhs.uk or email prac.health@nhs.net or call 0300 0303 300 or text NHSPH to 85258 (PHP crisis line)
- Professional support Unit: visit Professional Development | London (hee.nhs.uk) or email psu.lase@hee.nhs.uk
- BMA counselling and peer support service: visit Counselling and peer support for doctors and medical students (bma.org.uk) or call 0330 123 1245
- Resuscitation Council UK Responder Wellbeing https://www.resus.org.uk/responder-wellbeing
- Help for the Helpers: Vicarious-traumatisation.com
- Cavell nurses trust: visit www.cavellnursestrust.org/get-help or call 01527595999
- Samaritans: visit Samaritans.org/how-we-can-help-you or call 116 123 24hr Freephone

Appendix 3 – Links to additional resources for staff facilitating debriefs

- Group Psychological 'Debriefs' – Practice guidance for post-event team reflection (PETR) following distressing events at work; Association of Clinical Psychologists UK, July 2023

- NeoLeaP – Neonatal Unit Support Resource: Support during and after difficult local or national incidents or events, Neonatal Leads for ~Psychological Practice, August 2023

- PEARLs Healthcare debriefing tool - Promoting Excellence and Reflective Learning in Simulation (PEARLS). Development and Rationale for a Blended Approach to Health Care Simulation Debriefing (2015) PEARLS Debriefing Tool - Debrief2Learn

Appendix 4 – PITSTOP debriefing tool for Hot Debriefs

PITSTOP DEBRIEF FRAMEWORK

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Bringing the team together for a 5 – 10 minute supportive conversation following a difficult event.

PIT

PAUSE for a moment as a team (include all those who have been involved or could benefit). Agree how much time we have today.

INTENTION Share the aim of this PITSTOP (e.g., to pause and reflect on a challenging situation and check in with the team).

TALK THROUGH GROUND-RULES (attending or sharing is voluntary, what we discuss must remain confidential, the aim is for us to leave feeling connected and supported).

STOP

ummarise what has been happening

Facilitator summarises what's been happening (focusing on facts, not judgements). Facilitator then in others in the team to share their involvement in what's happened and their own perspectives. Allow individual experiences and feelings to be expressed and come back to the ground rules as needed.

hings we have been doing well

Facilitator to help colleagues to identify things that have been going well in how they have worked together during the situation. Highlight strengths and resources within the team, and what has made a difference so far today and can help the team moving forward.

ther things we are needing right now

Is there anything that we are needing right now (as individuals or as a team) that will support us with today's shift (practical, emotional)? Validate what is shared and encourage a climate where colleagues feel safe to speak up, recognising there will be individual differences in how people respond and cope.

romote wellbeing moving forwards

Facilitator to invite actions that promote self care and team care, recognising individual differences in how people respond and cope. Remind team it's normal to feel a range of thoughts and feelings when doing work that matters. Ask if there is anything further that can be done to promote wellbeing as a team, signpost to additional support where required. Remind team that resilience is between us not just within us.

Finish by signposting team members to the Staff Psychology Service if further support is needed. Psychological debriefs can also be requested by contacting:

THH Staff Psychology Service: thh.staffpsychology@nhs.net

Adapted from StartWell>EndWell

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